

PATIENT CLINICAL DETAILS

Physician Medical Record — Synthetic Clinical Data

1. Patient Information

Field	Information
Patient Name	Robert Anderson
MRN / Patient ID	MRN-202600391
Date of Birth	March 18, 1966
Age	60 years
Gender	Male
Insurance/Payer	Synthetic Health Plan B

2. Chief Complaint

Field	Information
Chief Complaint	Persistent nonproductive cough with intermittent throat irritation and mild chest discomfort associated with prolonged coughing.

3. History of Present Illness

Field	Information
Onset	Gradual onset approximately 4 weeks ago.
Duration	Approximately 4 weeks.
Relevant Injury or Condition	Persistent respiratory symptom without a reported acute chest injury.
Symptoms	Nonproductive cough occurring intermittently throughout the day, throat irritation, and mild chest discomfort during prolonged coughing episodes.
Progression	Symptoms have persisted without complete resolution and remain bothersome despite supportive measures.
Previous Attempts at Treatment	The patient has used increased oral hydration and an over-the-counter cough suppressant with incomplete symptom relief.

4. Symptoms

Field	Information
Symptoms	Persistent dry cough, intermittent throat irritation, mild chest discomfort after repeated coughing, and occasional fatigue associated with disrupted rest. No hemoptysis is reported.

5. Pain / Severity Evidence

Field	Information
Severity	Mild to moderate symptom burden based on reported frequency and persistence.
Pain Score	Mild chest discomfort rated approximately 2/10 during prolonged coughing episodes.

Field	Information
Severity Evidence	The cough has persisted for approximately four weeks despite conservative symptomatic measures. The patient remains independently ambulatory and able to perform routine daily activities.
Clinical Observations	Patient is afebrile at the current visit, oxygen saturation is 98% on room air, and there is no observed respiratory distress.

6. Physical Examination

Field	Information
General	Alert, oriented, and in no acute distress.
Vital/Respiratory Observation	Afebrile; oxygen saturation 98% on room air; respiratory effort unlabored.
Chest/Lungs	Clear breath sounds bilaterally without focal wheezing, crackles, or rhonchi.
Cardiovascular	Regular rate and rhythm on examination.
Chest Wall	No focal chest-wall tenderness at rest; mild discomfort reported after prolonged coughing.
Neurological	Grossly intact; no acute focal deficit observed.

7. Functional Impairment

Field	Information
Walking	Independent walking without limitation from the current symptoms.
Standing	Able to stand independently; no significant standing limitation reported.
Climbing Stairs	Able to climb stairs independently, although prolonged exertion may trigger coughing.
Working	Able to perform routine work activities, with occasional interruption from coughing episodes.
Daily Activities	Independent with routine activities of daily living; sleep and concentration may be intermittently disturbed by persistent coughing.

8. Diagnosis

Field	Information
Diagnosis Description	Chronic cough / persistent cough.
ICD-10-CM Code	R05.3

9. Previous Treatment

Treatment Name	Duration	Response / Outcome
Supportive Hydration Measures	3 weeks	Improved throat dryness but did not resolve the persistent cough.
Over-the-Counter Cough Suppressant	3 weeks	Partial temporary relief with continued intermittent coughing.

10. Previous Procedures

Field	Information
Procedure History	No previous procedures related to the current respiratory complaint are documented.

11. Diagnostic Tests / Imaging

Field	Information
Current Diagnostic Findings	No prior chest imaging for the current episode is documented in the available clinical record.
Clinical Observation	At the current visit, oxygen saturation is 98% on room air and lung auscultation is clear bilaterally.

12. Clinical Assessment

Field	Information
Assessment	Robert Anderson presents with a persistent nonproductive cough of approximately four weeks' duration despite supportive measures and over-the-counter symptomatic treatment. He is afebrile, maintains normal oxygen saturation on room air, and has clear bilateral breath sounds without respiratory distress. Further clinical evaluation is warranted to assess the persistent respiratory symptom.

13. Clinical Notes

Field	Information
Clinical Note	August 17, 2026 — Patient evaluated for persistent dry cough. Symptoms have continued for approximately four weeks despite hydration measures and an over-the-counter cough suppressant. No hemoptysis or acute respiratory distress reported. Patient remains independent in routine activities.

14. Provider Information

Field	Information
Treating Provider	Emily Johnson, MD
Provider Specialty	Internal Medicine
Facility Name / Type	Primary Care Clinic

15. Physician Certification

Field	Information
Certification	I certify that the information documented above reflects the patient's reported history, clinical findings, examination, and treatment history to the best of my knowledge.
Physician	Emily Johnson, MD
Date	August 17, 2026

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